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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°169/MVE-BDBV/30/10/2026

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HIGHLIGHTS (last 24 hours)

Sixty-three (63) new confirmed cases and 32 deaths (23 community deaths and 9 in Ebola Treatment Centers) of Ebola Virus Disease (EVD) were reported in the provinces of Ituri (34), North Kivu (25), and Haut-Uélé (4). A total of 439 new samples were analyzed, with a positivity rate of 14.4% for Bundibugyo virus. No new health zones were affected in the last 24 hours.

Data harmonization work was carried out on October 29, 2026, between the Data Management team and the SGI MVE BDBV provincial team in Ituri. This situation impacts the reported data for this province across 97% of health zones. As a result, the total number of deaths has increased from 3,436 to 3,750, bringing the overall total to 5,372.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
10414	5372	51.6%	1002	2,803	85.8%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi

67 out of 178 health zones affected (37.6%)

Summary of key points from the last 24 hours

At the end of the day on October 30, 2026, sixty-three (63) **new confirmed cases**
Cases of Ebola virus disease (EVD) have been reported in three provinces : **Ituri (34 cases), North Kivu (25 cases), and Haut-Uélé (4 cases).** Among these cases, **there have been 23 community deaths.** were reported **(12 in Ituri, 8 in North Kivu and 3 in Haut-Uélé).** In addition, **9** confirmed deaths were recorded in the CTEs (7 in Ituri and 2 in North Kivu), bringing the total number of confirmed deaths reported during the period to **32 .**

Furthermore, 15 patients **(12 in Ituri, 2 in North Kivu and 1 in Haut-Uélé)** were declared cured after obtaining two negative results in the control test for Ebola Virus Disease.

Since the start of the epidemic, the DRC has recorded **10,414 confirmed cases,** including **5,372 deaths,** representing a **case fatality rate of 51.6** %. **Ituri** province remains the epicenter of the epidemic, accounting for **70.9% of all reported confirmed cases** and **54.0% of new confirmed cases** notified nationwide, while North Kivu accounts for **39.7% of new cases** in the last 24 hours.

No new health zones have been affected in the last 24 hours.

Thus, 67 out of 178 health zones (37.6%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 30, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	34	7380	3,769	51.1%	28/36 (77.8%)
North Kivu	25	2461	1,366	55.5%	17/34 (50.0%)
Haut-Uélé	4	457	203	44.4%	7/13 (53.8%)
Tshopo	0	92	27	29.3%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	13	4	30.8%	4/11 (36.4%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
North Ubangi	0	6	1	16.7%	2/11 (18.2%)
Total	**63**	**10,414**	**5,372**	**51.6%**	**67/178 (37.6%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 30, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7380**	**3769**	**51.1%**	**34**	**12**	**7**	**19**
Adja	11	3	27.3%				0
Aru	25	3	12.0%				0
Ariwara	11	4	36.4%				0
Aungba	12	7	58.3%				0
Bamboo	243	121	49.8%				0
Boga	3	2	66.7%				0
Bunia	1873	938	50.1%	9	5	1	6
Damascus	40	30	75.0%				0
Drodro	19	12	63.2%				0
Fataki	94	46	48.9%				0
Gethy	11	7	63.6%				0
Kambala	4	2	50.0%				0
Kilo	36	11	30.6%	1	0	0	0
Komanda	190	138	72.6%				0
Lita	292	177	60.6%	1	1	0	1
Logo	16	5	31.3%				0
Lolwa	43	24	55.8%	1	1	0	1
Mahagi	4	4	100.0%				0
Mambasa	84	47	56.0%	2	1	2	3
Mandima	484	302	62.4%	4	2	3	5
Mangala	443	227	51.2%	1	0	0	0
Mongbwalu	695	368	52.9%	3	1	0	1
Nia-Nia	368	259	70.4%	1	0	0	0
Nizi	846	494	58.4%	5	1	1	2

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Nyankunde	154	57	37.0%				0
Rimba	25	12	48.0%	1	0	0	0
Rwampara	1267	433	34.2%	5	0	0	0
Tchomia	87	36	41.4%				0
North Kivu	**2461**	**1366**	**55.5%**	**25**	**8**	**2**	**10**
Blessed	690	455	65.9%	13	7	0	7
Good	26	9	34.6%	1	0	0	0
Butembo	342	199	58.2%	1	0	0	0
Goma	1	0	0.0%				0
Kalunguta	96	43	44.8%				0
Katwa	676	413	61.1%	3	0	1	1
Kayna	3	2	66.7%				0
Kyondo	46	22	47.8%				0
Lubero	43	15	34.9%	2	0	0	0
Mabalako	116	39	33.6%	1	0	0	0
Alimbongo	6	4	66.7%	1	1	0	1
Manguredjipa	16	5	31.3%				0
Masereka	41	13	31.7%				0
Musienene	161	77	47.8%	1	0	0	0
Mutwanga	55	14	25.5%	0	0	1	1
Oicha	87	38	43.7%	2	0	0	0
Vuhovi	56	18	32.1%				0
Upper Uele	**457**	**203**	**44.4%**	**4**	**3**	**0**	**3**
Boma Mangbetu	46	19	41.3%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	113	53	46.9%				0
Pawa	113	65	57.5%	2	2	0	2
Rungu	7	4	57.1%	1	1	0	1
Wamba	174	60	34.5%	1	0	0	0
Tshopo	**92**	**27**	**29.3%**	0	0	0	0
Bafwasende	18	3	16.7%				0
Kabondo	20	2	10.0%				0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	37	14	37.8%				0
Mangobo	10	7	70.0%				0
Tshopo	5	0	0.0%				0
Wanie -Rukula	1	1	100.0%				0
South Kivu	**3**	**1**	**33.3%**	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0
North Ubangi	**6**	**1**	**16.7%**	0	0	0	0
Wapinda	3	1	33.3%				0
Yakoma	3	0	0.0%				0
Total	**10414**	**5372**	**51.6%**	**63**	**23**	**9**	**32**

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* An exercise to harmonize undisclosed data was carried out on October 29, 2026 between the Data Management pillar and the provincial SGI of Ituri. This exercise led to adjustments in the data reported in the province.

Table 3. Status of alerts notified by province for October 30, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated ed	Number of suspects No investigated ed	Number of suspects investigated and transferred At CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased			
Haut-Uélé	93	3	96	15	3	78	0	15	3	6
Bas Uélé	194	4	198	16	4	178	0	20	0	0
Ituri	1,180	52	1,232	92	52	1088	0	142	2	76
North Kivu	2044	85	2,129	199	75	1845	10	188	86	129
South Ubangi	47	0	47	1	0	46	0	1	0	1
North Ubangi ND 100		ND	ND	ND	ND	ND	ND	ND	ND	ND
Tshopo		0	100	8	0	92	0	0	8	0
Total General	**3,658**	**144**	**3,802**	**331**	**134**	**3,327**	**10**	**366**	**99**	**212**

Figure 1. Distribution of confirmed EVD cases by date of symptom onset and outcome from April 20 to

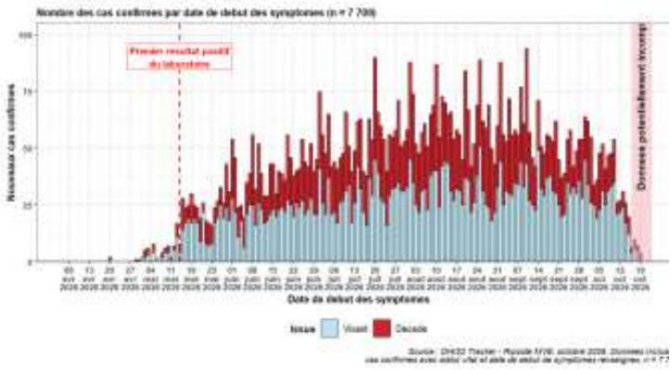
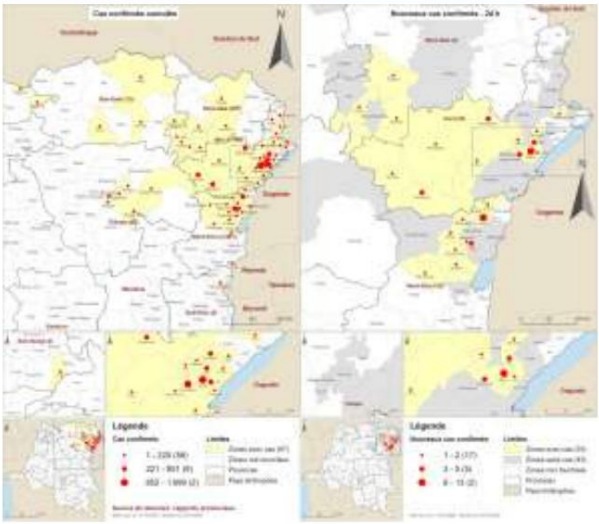


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars; continuation in Kisangani, on October 30, 2026, of the work of the Intra-Action Review (IAR) of the response against the 17th Ebola outbreak, preceded by a security briefing of the participants and a systematic screening with hand hygiene of approximately 250 participants.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

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As of October 30, 2026, **3,802 alerts** had been reported in the six provinces that submitted their data (Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu, Sud-Ubangi, and Tshopo), including **3,658** alerts of living persons and **144 of deaths ;** all were verified (100%). Of these, 465 (12.2%) were validated as suspected cases and 3,337 (87.8%) were invalidated. Of the 465 validated suspected cases, 366 (78.7%) were investigated and 212 were transferred to COVID-19 treatment centers (CT/CTE); 99 suspected cases remain uninvestigated, including 86 in Nord-Kivu, 8 in Tshopo, 3 in Haut-Uélé, and 2 in Ituri (Table 3). Active search covered 50 out of 68 planned sites (73.5%) in 6 health zones in Haut-Uélé and 77 out of 81 structures (95.1%) in Bas-Uélé.

In total, 37,674 contacts were under follow-up in the six provinces that submitted their data; 32,306 were seen in the last 24 hours, representing a follow-up rate of 85.8%, below the required threshold of 95.0% but an improvement compared to the previous day (84.5%). By province, the follow-up rate is 99.9% in Bas-Uélé (783/784), 90.8% in Ituri (13,768/15,155), 89.3% in Sud-Ubangi (109/122), 84.9% in Haut-Uélé (1,906/2,245, of which only 68.9% in Boma Mangbetu and 71.1% in Wamba), 81.7% in Nord-Kivu (15,000/18,357) and 73.2% in Tshopo (740/1,011), where the health staff strike and attacks against teams still disrupt follow-ups (Table 4).

Table 4. Status of contact tracing by province, as of October 30, 2026

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	15,155	13,768	90.8%
Haut-Uélé	2,245	1906	84.9%
North Kivu	18,357	15,000	81.7%
Tshopo	1011	740	73.2%
Bas-Uélé	784	783	99.9%
South Ubangi	122	109	89.3%
North Ubangi	N / A	N / A	N / A
Total	37,674	32,306	85.8%

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

For October 30th, ten (10) alerts were notified: 5 in Ituri, 3 in Bas-Uélé, 1 in Tshopo, and 1 in South Kivu. In Ituri, of the 4 validated live alerts, only one was transferred to the Nizi CTE (25%; 1/4), as the three travelers intercepted at the Bela PoC (Mandima Health Zone) refused transfer, and the body of a 33-year-old man intercepted at the Mudzi Bala PoC (Bunia Health Zone) was swabed (100%; 1/1), with the result pending. In Tshopo, the live alert from the Bangboka airport PoE was validated, isolated, investigated, and referred to the Simi-Simi RVA CS (100%; 1/1); In Bas-Uélé, the 3 alerts from PoC PK2 (ZS of Poko) were invalidated after verification; in Sud Kivu, the body of a 76-year-old woman who died in Burundi was notified to the PoE of Kavimvira (ZS of Uvira); in Nord Kivu, a hut and its handwashing device were set on fire at PoC Maboya (ZS of Kalunguta) on the night of October 29-30 (Table 5).

Table 5. Status of key indicators for PoE/PoC by province, as of October 30, 2026

Indicators	Ituri	North-Kivu	South Kivu	Tshopo	High-Uele	Bas-Uélé	South-Ubangi	Overall

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Completeness of the of reports	97% (60/62)	95% (20/21)	56% (5/9)	100% (18/18)	75% (9/12)	73% (16/22)	60% (3/5)	87.9% (131/149)
Strategic PoC/PoE	1	N / A	N / A	N / A	N / A	N / A	N / A	1
Number of past travelers	161 528 100 862 8 957			21,940 16,252		6,873	2,237	318,649
Proportion of travelers screened	96%	98%	99%	99%	99%	98%	85%	97%
% of travelers who washed their hands	96%	98%	99%	98%	99%	99%	85%	97%
% of travelers made aware	100% 98%		100%	100% 100% 89%			100%	99%
Number alerts notified	05	N / A	01	01	00	03	00	10
Number of validated live alerts	04	N / A	00	01	00	00	00	5
Number of lifeless bodies intercepted	01	N / A	01	00	00	00	00	2
% of suspected cases transferred to CT/CTE	25% (1/4)	0%	0%	100% (1/1)	0%	0%	0% 40% (2/5)	
% of lifeless bodies swabed/ secured	100% (1/1)	0% 0% (0/1) 0%			0%	0%	0% 50% (1/2)	
% of suspected cases investigated within 24 hours	50% (2/4)	0%	0%	100% (1/1)	0%	0%	0% 60% (3/5)	

2.2.2. Challenges

In North Kivu, the fire at the Maboya PoC (PoC) comes on top of the closure of the Butuhe PoC since September 26, 2026. Alerts from the PoCs are not being addressed promptly by the alert units, and the pillar lacks resources.
communication; in Ituri, three out of four live alerts refused their transfer to the Bela PoC, the providers of the Mandima PoCs still need training, there is a lack of mobility for supervision in Bunia and Rwampara and September salaries have not been paid; **in Bas-Uélé,** the completeness remains low (73%; 16/22) and the PoE/PoCs of the border ZS of **North-Ubangi** and CAR still need training; **in Sud-Ubangi,** the Libenge and Dongo posts did not report and 15% of travelers refused screening (335/2,237), including 23.4% at the Akula PoC.

2.3. Laboratory

2.3.1. Main actions

- **Ituri:** 34 new positive results (22 living and 12 dead) out of 206 new samples analyzed (positivity: 16.5%).
- **North Kivu:** 25 new positive results (17 living and 8 dead) retained by surveillance; the laboratories of Beni and Butembo analyzed 208 new samples (positivity: 12.0%) and 38 follow-up re-samples.
- **Tshopo:** Six samples from living individuals were analyzed, all negative (positivity: 0%).
- **Haut-Uélé:** 4 new positive results (1 live and 3 deaths) out of 19 new samples analyzed at the Isiro laboratory (positivity: 21.1%).

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 15 rings opened out of 24 (62.5%), 35 households decontaminated out of 39 (89.7%) and 7 out of 7 FOSA (100%), mainly in Rwampara, Nizi, Bunia and Mangala; 60 EDS carried out out of 83 alerts (72.3%) and 56 bodies swabed out of 69; training of clinical providers in the Fataki health zone in Ebola surveillance, obstetric IPC and SONU.
- **North Kivu:** 23 rings opened out of 27 (85.2%), 6 households decontaminated out of 15 (40.0%) and 20 ESS out of 22 (90.9%); 156 ESS follow-ups, 282 providers briefed, 304 DLM functionality follow-ups, 3 triage units installed and 6 score card assessments (average score: 37.2%); 56 schools equipped with PCI-WASH kits in Kyondo (38, CAUB/UNICEF support) and in Vuhovi (18, World Vision support); 46 EDS carried out and 51 bodies swabed out of 51 alerts.
- **Tshopo:** decontamination of 2 households around the 90th and 91st confirmed cases and of a restaurant (ZS of Makiso-Kisangani); provision of IPC inputs to 3 ESS in Kabondo and 6 replacement mattresses to

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households and ESS around the cases; follow-up of 13 ESS and briefing of 197 providers in 5 ZS; reassessment of the Polyclinique du Millénaire (score: 58.1% against 51.6% initially).

- **Haut-Uélé:** 2 out of 2 rings open (100%), 20 out of 20 households decontaminated (12 in Boma Mangbetu, 4 in Isiro and 4 in Pawa) and 1 out of 1 ESS in Pawa; provision of IPC kits to 3 affected schools in Isiro (EP Tely, EP Kasongo and Collège Les Étoiles); 3 ESS assessed in Wamba and 16 nurses briefed on triage and standard precautions; 5 EDS carried out on 5 alerts and 4 bodies swabés.

- **Bas-Uélé:** decontamination of the isolation room of the Dakwa General Referral Hospital; monitoring of the ESS of 5 ZS and briefing of 21 service providers and 5 hygienists on the wearing of PPE, waste management, sorting and preparation of chlorinated solutions; scorecard evaluation of the Koteli General Referral Hospital (51%); 1 body swabé in Ganga, no EDS carried out; in Sud-Ubangi, the service providers of the Bulu ZS were trained on IPC.

2.4.2. Challenges.

- **In North Kivu,** none of the 6 decontaminated households received a kit (0/6), 4 rings remain closed, mainly in Katwa, Lubero, and Butembo, 2 people with primary AIDS were infected, and 42 EDS (Emergency Discharge Summary) tests were postponed to the following day. In Ituri, 9 rings remain closed, the Infection prevention and control (IPC) teams remain insufficient in Mangala, Mandima, Damas, and Mambasa; IPC supplies are lacking in Mandima, Niania, and Mambasa; and 13 swab failures were recorded. In Bas-Uélé, only 130 out of 632 service providers (20.6%) are trained in IPC-WASH, 16 out of 20 priority health facilities (HHFs) lack functional triage, and no WASH partners support case management activities. **In Haut-Uélé,** only 4 out of 7 health zones (HHZs) submitted their data, and IPC kits are lacking for HHFs; **in Tshopo,** only 49 out of 107 HHFs (45.7%) have a triage system, and provider training remains limited due to a lack of funding.

2.5. Holistic care

2.5.1. Main actions

In Ituri, 78 new admissions (all suspected cases) were recorded, compared to 98 discharges, including 12 recoveries and 5 discharges without medical advice. Thus , 401 patients are hospitalized, representing an occupancy rate of 38%; 377 out of 391 patients (96.4%) received hot meals, and the Kasenyi Ebola Treatment Center (ETC) did not report due to a strike by healthcare providers.

In North **Kivu,** 76 new admissions (7 confirmed and 69 suspected) were recorded, compared to 116 discharges, including 2 recoveries, 110 non-cases, 2 confirmed deaths, and 2 discharges without medical advice. A total of 506 patients (176 confirmed and 330 suspected, including 92 children under 5) are currently receiving care, 145 of whom are outside of Ebola Treatment Centers (ETCs), representing an occupancy rate of 88.0% (361/410) for ETCs.

- **In Tshopo,** no new admissions were recorded against 4 discharges of non-cases; 15 patients (12 confirmed and 3 suspected) are hospitalized at the CTE Météo (occupancy rate: 41.7%; 15/36) and all received hot meals, as did the 51 PPLs followed.

In Haut-Uélé, 11 new admissions (2 confirmed and 9 suspected) were recorded, compared to 2 discharges (1 recovered in Boma Mangbetu and 1 non-case in Isiro); 32 patients (15 confirmed and 17 suspected) are hospitalized (occupancy rate: 19.8%; 32/162). In **Bas-Uélé,** 10 new suspected cases were isolated in the health centers (Viadana, Titule, Ganga, and Poko) with no discharges, bringing the number of isolated suspected cases to 48 due to the lack of a functioning treatment center.

2.5.2. Challenges

In Ituri, the blood shortage persists, 14 patients in Mandima have not received nutrition, 40 patients are critically ill, and capacity remains low in Nyankunde, Mambasa, Kilo, Lolwa, Mandima, and Biakato. In North Kivu, Ebola Treatment Centers (ETCs) remain near capacity (88.0%), 145 patients are being treated outside of ETCs, and 19 are critically ill. In Tshopo, a building at the Bafwasende ETC was burned down, and two confirmed escapees remain missing. In Haut-Uélé, staff demotivation, insufficient nutritional supplies, and poor network coverage at ETCs persist. In Bas-Uélé, the standardized ETC in Buta is still under construction, and none of the 48 isolated suspected cases are receiving nutritional support.

2.6. Vaccination

2.6.1. Main actions

In Ituri, vaccination of people living with HIV (PLHIV) using Ervebo (MSF support) and research on its effectiveness against BDBV-related Ebola virus disease (BBDV) are continuing: 134 people were vaccinated during the day (60 women and 74 men), bringing the total to 2,215 (925 women and 1,290 men). In North Kivu, after receiving 2,000 doses of Ervebo, 200 PLHIV were vaccinated in the Beni health zone, bringing the total to 800 out of 7,105 expected (11.3%); in Bas-Uélé, compassionate vaccination has ended in the Ango and Dakwa health zones, with 1,943 people vaccinated out of 13,202 targeted (14.7%).

2.6.2. Challenges

In Bas-Uélé, the start-up of the UCCs is still not effective and the support of field actors is lagging behind; in North Kivu, the coverage of the PPLs in Beni remains low (11.3%) and 1 non-serious MAPI has been notified; in Tshopo, the activation of the continuity plan of the PEV is still awaited.

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2.7. Continuity of Care

2.7.1. Main actions • In Tshopo, the evaluation

of the Tshopo 1 Health Center, the La Promédie Medical Center (Mangobo) and the Maternity Without Risk Medical Center (Makiso-Kisangani) revealed an ineffective triage area, the absence of a partogram and low staff vaccination against Ebola (0% to 37.5% according to the ESS), and recovery plans for the Bethany Health Center and the Maison Médicale Polyclinic were developed; **in North Kivu,** the baseline assessment of the Anuarite Health Center revealed the absence of triage, shortages of ORS-Zinc, PEP kits and nutritional inputs, and only 9 out of 16 providers (56.3%) trained in IPC; In Ituri, the Bunia 1 clinic was resupplied with medicines and inputs (WHO support), the ESS in Mandima was assessed and the providers in Fataki were trained in SONU (MEDAIR support).

2.7.2. Challenges

Insufficient supply of labile blood products in Ituri, where needs are estimated at 4,900 bags per month, including 1,000 for the Bunia and Rwampara health zones alone; decline in the use of maternity, IMCI and vaccination services (BCG, Penta 1 and Penta 3) in the visited health areas and non-conducted CSSE assessments in the Lubunga health zone (Tshopo); absence of triage, disruption monitoring dashboard and absenteeism contingency plan at the Anuarite health center (North Kivu); no data transmitted by Haut-Uélé, Bas-Uélé and Sud-Ubangi.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

• **In Ituri,** 22 out of 29 feedbacks (75.9%) and 10 out of 11 incidents (90.9%) were resolved within 48 hours and 399 community alerts, including 155 deaths, were notified; the CREC facilitated the pre-listing of 962 contacts around 22 confirmed cases, the decontamination of 19 out of 19 households, the completion of 19 out of 25 EDS (76.0%) and the transfer of 91 out of 92 suspects; 21,452 home visits reached 80,434 people, 832 educational talks reached 9,658 people, 88 community debates reached 1,721 people and 107 mass sessions reached 2,701 people; 7 advocacy efforts involved religious authorities in Komanda and Rwampara, 28 spots were broadcast on 36 radio stations and 10 programs organized; in Mambasa, 3 healed individuals were reintegrated into their community and, in Bunia, a radio denial was broadcast against the rumor of the end of the response.

• **In North Kivu,** 17,993 home visits reached 67,993 people, 278 educational talks reached 7,723 people, 64 mass sessions reached 7,721 people and 74 briefings reached 958 people; 412 alerts out of 650 (63.4%) were notified, 24 feedbacks out of 37 (64.9%) and 7 incidents out of 22 (31.8%) were resolved within 48 hours and 2,810 people out of 4,750 (59.2%) were engaged in community actions; Community trainers and 30 school principals were trained in Musienene, 16 briefing sessions of local leaders were held in Kyondo and, in Mabalako, the CREC overcame a family reluctance, allowing a sick person to be taken to the CTE, and calmed about 200 young people opposed to an EDS.

In **Haut-Uélé,** 285 home visits reached 2,509 people, 77 educational talks reached 1,530 people, and 8 mass activities in schools reached 3,477 people; the two feedback requests and the community incident in Bolebole were resolved, the U-Report youth engagement workshop in Isiro was prepared, and CREC supported the supervision of mosquito net distribution in Isiro. **In Bas-Uélé,** 515 home visits reached 2,484 people, 7 mass awareness campaigns reached 1,596 people, 7 educational talks reached 156 people, 1 community debate reached 125 people, and 2 radio programs were broadcast; the 14 community alerts and 4 feedback requests were resolved, and a rumor about vaccination was addressed in five schools in Dakwa.

• **In Tshopo,** 497 home visits reached 2,752 people, 11 mass awareness campaigns reached 14,538 people, including 958 students from the Sainte Vierge Marie school complex (Kabondo), and 5 educational talks reached 251 people; 6 interactive programs were hosted, 63 posters and 186 advice cards were distributed, 2 out of 3 refusals were lifted and 3 out of 3 feedback issues were resolved; the provincial CREC actors and those from the 7 ZS hotspots were trained in infodemic management.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

• **In Ituri,** 26 out of 28 confirmed cases (92.9%), 67 out of 122 confirmed cases and 123 out of 190 suspected cases under follow-up, as well as 48 out of 75 new suspected cases, have been taken care of, 14 out of 15 separated and orphaned child households (93.3%) have been accompanied, 67 children are being followed up, 8 out of 12 have been reintegrated and 46 results have been announced, including 26 positive results; 246 accompanying persons and 79 PPLs have been supported.

• **In Haut-Uélé,** 1 new confirmed case, 11 confirmed and 4 suspected cases under follow-up as well as 3 new suspected cases were taken care of in Isiro, Pawa and Wamba, 15 separated children are being followed in the community and 28 accompanying persons and 40 PPLs have been supported.

• **In Tshopo,** the 12 confirmed cases under monitoring and the new suspect have been taken care of, the resistance notified around a confirmed case of AS Muungano (Kabondo) has been lifted, 14 negative results have been announced and 311 people, including 190 students from EP1 Lumbulumbu (Mangobo) and 70 inspectors and teachers from the Tshopo health zone, have benefited from psychoeducation.

• **In Bas-Uélé,** no SMSPS interventions were reported for the day; SMSPS data from North Kivu were not transmitted.

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2.9.2. Challenges

In Ituri, only 15 out of 28 affected health zones (53.6%) are covered, and coverage remains incomplete around the last cases in Mangala (1/2). In Haut-Uélé, no daycare centers are available for separated children in Isiro, Pawa, Wamba, and Boma Mangbetu; community health workers lack transportation, and nutritional kits for weaned infants are insufficient. In Tshopo, community health workers in the Wanie-Rukula health zone are not trained, and the health pillar lacks communication units and transportation. In Bas-Uélé, psychosocial support for confirmed cases is lagging.

2.10. Logistics

2.10.1. Main actions

- **In North Kivu,** IPC inputs (boots and household gloves) were delivered to the Mabalako Health Zone (Mangina), therapeutic milk to the nutritional support pillar and tents and chairs to the coordination; WASH-ICP inputs intended for schools, ESS and households of cases are being unloaded in Beni and Butembo (UNICEF support) and providers of the different pillars have been deployed in the hotspot Health Zones.

- **In Ituri,** for health products and medicines (PSM), the Bunia 1 clinic has been resupplied with medicines and inputs (WHO support), but the blood stock shortage persists; **in Tshopo,** 3 ESS in Kabondo have been provided with PCI inputs and 6 replacement mattresses given to households and ESS around the cases, and compliance with the free provision of essential generic medicines provided by the UG-PDSS is monitored in the ESS.

- **In Haut-Uélé,** 3 out of 5 planned vehicles have been received (MSP/UG-PDSS), 460 litres of fuel have been received from FHI 360 for Wamba, Pawa and the coordination, and response equipment has been delivered to the health zones of Boma Mangbetu and Watsa after the inventory of priority input stocks; **in Bas-Uélé,** 5 motorcycles have been received as a donation from WHO, the construction of the CTE at the Buta General Referral Hospital continues (50% of the work) and the packaging of PCI and PEC inputs for the health zones is in preparation, but diesel, computers and printers are still lacking.

2.11. Security

2.11.1. Main actions

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- **In Tshopo,** a building at the Bafwasende CTE was set on fire and a security briefing was organized for participants in the Kisangani RIA; in North Kivu, the PCI and EDS teams were escorted for the decontamination of 2 medical centers and 4 households and for a burial in Masiani, while a hut at the Maboya PoC (Kalunguta Health Zone) was set on fire; in Haut-Uélé, the head nurse and the resident nurse at the Beach Bolebole health post were threatened by members of the community; in Bas-Uélé, the security situation related to the strike of service providers is being monitored.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

In North Kivu, 38 local leaders from Beni (supported by Care International) and 22 registered nurses from Musienene were briefed on the PSEA and reporting via 495555/141, and 8 members of the response team from Uganda were sensitized in Mutwanga; in Haut-Uélé, 12 village chiefs from the Nangazizi AS (Rungu) and 54 participants in the tree day in Wamba were sensitized on the PSEA and the reporting circuit; in Tshopo, 19 staff members from CAJCO (Mangobo) were sensitized on the PEAHS and zero tolerance.

2.12.2. Challenges

- Coverage of affected areas remains low: in North Kivu, only 3 out of 34 health zones (8.8%) know the reporting mechanism and only one partner (Care) reports its activities; in Haut-Uélé, none of the 14 partners report their PSEA activities and awareness tools in French and Lingala are insufficient; in Tshopo, collection tools are lacking and the community complaint management mechanism is not yet installed in all the health zones of Kisangani; only 3 out of 8 health districts (37.5%) reported their PSEA activities in the last 24 hours.

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KEY MESSAGES

No new health zones have been affected in the last 24 hours.

67 out of 178 health zones (37.6%) remain affected in eight provinces.

Work on data harmonization was carried out on October 29th.

2026, between the Data Management pillar team and the SGI MVE BDBV team

Ituri provincial government. This situation has an impact on the number of data points

this province across 97%.

Sixty-three (63) new confirmed cases were reported in three provinces, including 34 in Ituri, 25 in North Kivu, and 4 in Haut-Uélé. Transmission remains particularly

marked in the health zones of Beni (13 cases), Bunia (9 cases), Nizi (5 cases) and Rwampara (5 cases).

Of the 32 confirmed deaths reported, 23 were community deaths (12 in Ituri, 8 in North Kivu, and 3 in Haut-Uélé). The contact tracing rate stands at 85.8% (32,306/37,674), up from the previous day (84.5%) but still below the 95% threshold, with the lowest rates in Tshopo (73.2%), where the strike...

Health personnel are disrupting follow-up care, and in North Kivu (81.7%). The situation highlights the

There is a need to strengthen early detection, contact tracing, and dignified and safe burials.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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EBOLA PRESIDENTIAL FORCE TASK 17

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